

Policy on Safe and Rational Prescription of Medications

«Hospital Name»

Document control

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Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Medications at «Hospital Name» are prescribed rationally and meet defined minimum requirements every time, drug allergies are checked before every prescription, verbal orders follow a documented safe process, and medications are reconciled at every transition in a patient's care.

1. Purpose

This policy explains how «Hospital Name» ensures rational prescribing, applies minimum prescription requirements, checks drug allergies and adverse drug reactions before prescribing, reconciles medications at transitions of care, controls verbal orders, and audits prescriptions for safety.

The World Health Organization defines rational use as ensuring "patients receive medications appropriate to their clinical needs, in doses that meet their own individual requirements, for an adequate period of time, and at the lowest cost to them and their community" (WHO, Rational Use of Medicines, who.int/medicines/areas/rational).

This policy does not cover storage, dispensing, or administration of medications in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all doctors who prescribe medications, and to pharmacy and nursing staff involved in checking, dispensing, and administering them, at «Hospital Name».

3. Policy standards

«Hospital Name» prescribes medications in line with rational-prescribing good practice for both outpatients and inpatients, applies defined minimum requirements to every prescription, checks drug allergies and previous adverse reactions before prescribing, gives clinicians a mechanism to help avoid interactions and dosing errors, reconciles medications at every transition of care, controls verbal orders through a documented process, and audits prescriptions monthly for safety and rationality, acting on the findings where appropriate.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Do not prescribe outside of good practice and guidelines for rational prescribing — this applies to both outpatient and inpatient care — and do not allow clinicians to prescribe without being trained and sensitised on the rational use of medications.
2. Do not accept, dispense, or administer a prescription that fails «Hospital Name»'s determined minimum requirements: patient name, unique hospital number, drug name (generic composition, except for vitamin or mineral combinations), strength, dosage instruction, duration and total quantity, and the prescribing doctor's name, signature, and registration number. Do not use error-prone abbreviations, and write every prescription in capital letters.
3. Do not prescribe, or transcribe a prescription for action, without first ascertaining the patient's drug allergies and previous adverse drug reactions.
4. Medication reconciliation is mandatory at every admission, at every internal transfer between wards or departments, and at discharge; no patient episode may proceed to the next stage without documented reconciliation.
5. Do not act on a verbal medication order outside «Hospital Name»'s documented verbal-order process — including read-back, documentation at the time it is received, and countersignature by the ordering doctor within 24 hours.
6. A medication order audit using a representative sample shall be completed every calendar month; findings that warrant corrective or preventive action must be documented and actioned within the timeframe set by the Pharmacy and Therapeutics Committee.

5. What we do

5.1 Prescribe rationally

Prescribing follows good practice and guidelines for rational prescription of medications, across both outpatient and inpatient care. Clinicians are trained or sensitised on rational prescribing. The Code of Medical Ethics-2002, published by the Medical Council of India (now the National Medical Commission), is followed.

5.2 Meet minimum prescription requirements

Every prescription — whether inpatient, outpatient, or emergency — follows applicable national and international guidelines. At minimum, it includes:

1. the patient's name
2. the patient's unique hospital number
3. the drug name in generic composition, except for vitamin or mineral combinations
4. strength
5. dosage instructions
6. duration and total quantity
7. the prescribing doctor's name, signature, and registration number.

Error-prone abbreviations are not used, and prescriptions are written in capital letters. It is preferable to use a digital prescription system to reduce errors. Where a prescription has an error or is illegible, it is corrected with a single strikethrough, initialled, and rewritten.

Common error-prone abbreviations to avoid include "U" for units (misread as a zero, causing a 10-fold overdose), "QD"/"QOD" (mistaken for each other), a trailing zero as in "1.0 mg" (misread as 10 mg), and "MgSO4"/"MSO4" (confused with each other) — see the ISMP List of Error-Prone Abbreviations, Symbols, and Dose Designations (ismp.org).

5.3 Check for drug allergies and prior reactions

Drug allergies and any previous adverse drug reactions are ascertained before prescribing — during the initial consultation, or at any point during the patient's care. It is good practice to record this prominently in the medical record for both outpatients and inpatients.

5.4 Help clinicians avoid interactions and dosing errors

«Hospital Name» provides clinicians with a mechanism — electronic or physical — to help identify drug interactions, food-drug interactions, alcohol-drug interactions, therapeutic duplication, and dose adjustments.

5.5 Reconcile medications at transitions of care

Prescribed medications are checked for accuracy at three points:

1. at admission — whether direct or after admission from the emergency department
2. at transfer between wards or departments
3. at discharge.

It is preferable to also reconcile after a cross-consultation. Reconciliation is documented, and there is a system to communicate it effectively during handover.

5.6 Control verbal orders

Written guidance sets out who can give a verbal medication order, when it can be given, and how it is authenticated. Verbal orders are limited to urgent situations where immediate written or electronic communication is not practical, and their use is kept to a minimum. An approved list of formulary drugs that can be ordered verbally is maintained — defined either by what is included or what is excluded. The process includes read-back or repeat-back, and every verbal order is countersigned by the ordering doctor within 24 hours.

5.7 Audit prescriptions

Medication order and prescription audits check legibility and use of capitals, the appropriateness of drug, dose, frequency, and route, therapeutic duplication, drug and food-drug interactions and how they are avoided, dosage adjustment for renal or hepatic impairment, IV incompatibility, inappropriate dilutions or infusion durations, and adherence to this policy's other requirements. This happens at least once a month, using a representative sample. It is preferably done by a clinical pharmacologist or clinical pharmacist — where none is available, a trained multidisciplinary team can do it instead. It is preferable to audit prescriptions live, before dispensing.

5.8 Act on audit findings

Where appropriate, corrective or preventive action is taken based on the audit — ideally guided by root-cause analysis — and records of these actions are kept.

6. Stop-work authority

Do not prescribe (or transcribe a prescription for action) when drug allergies and previous adverse drug reactions have not been ascertained.

Do not act on a verbal medication order except through the organisation's documented verbal-order process (read-back, documentation, countersignature within the defined time).

Do not accept a prescription that fails the organisation's determined minimum requirements.

Stop-work applies to writing or acting on the unsafe order. Immediate life-saving medication in an emergency follows the documented emergency-prescription rules and is written up as soon as the patient is stable.

The person who stops tells the treating doctor and the Medication Safety Officer the same shift. Refusing an unsafe prescription is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for rational prescribing implementation; maintains the authorised-prescriber list; receives stop-work escalations.
Medication Safety Officer	Owns the prescription audit programme; brings audit findings and incidents to the Drug and Therapeutics Committee.
Treating doctors / prescribers	Follow rational-prescribing guidance and the Code of Medical Ethics-2002; apply minimum prescription requirements; check drug allergies before prescribing; follow the verbal-order and reconciliation processes.
Pharmacy In-Charge	Enforces minimum prescription requirements at the point of dispensing; holds the list of drugs orderable verbally; implements the verbal-order control process.
Nursing staff	Check prescriptions before administration; complete medication reconciliation at transitions of care.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

8. Quality monitoring

Monitoring area	What is monitored
Rational prescribing	Clinicians trained; Code of Medical Ethics-2002 followed; prescribing in line with rational-prescribing guidelines for outpatients and inpatients.
Minimum prescription requirements	All seven elements present (patient name, unique hospital number, generic drug name, strength, dosage instruction, duration and total quantity, prescriber name/signature/registration number); capital letters used; no error-prone abbreviations; non-compliant prescriptions held.
Drug-allergy check	Drug allergies and previous ADRs ascertained and documented before prescribing for every patient.
Prescribing assistance mechanism	Mechanism available and working — electronic or physical — for drug interactions, food-drug interactions, therapeutic duplication, and dose

	adjustments.
Medication reconciliation	Documented at all three transition points: admission, transfer between wards or departments, and discharge.
Verbal orders	Read-back performed and documented; countersignature by ordering doctor within 24 hours; verbal orders limited to urgent situations.
Prescription audit	Conducted at least once a month with a representative sample; all scope parameters covered; findings documented; CAPA recorded where appropriate.
Stop-work events	Stop-work events logged with trigger, action taken, and outcome.

9. Training and staff acknowledgement

All doctors who prescribe, and pharmacy and nursing staff involved in checking, dispensing, and administering medications, shall be familiar with the rational-prescribing guidance, minimum prescription requirements, allergy-check process, reconciliation requirements, verbal-order process, and stop-work authority in this policy.

I have read the Policy on Safe and Rational Prescription of Medications of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all treating doctors, pharmacy staff, nursing staff, the Medical Superintendent, the Medication Safety Officer, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
ADR	Adverse drug reaction
CAPA	Corrective and Preventive Action
DTC	Drug and Therapeutics Committee
IV	Intravenous
MCI	Medical Council of India
MOM	Management of Medication (NABH Hospitals chapter)
MSO	Medication Safety Officer
NABH	National Accreditation Board for Hospitals

	and Healthcare Providers
NMC	National Medical Commission

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.4.a	Commitment*	Sections 3 and 5.1 address rational prescribing for outpatients and inpatients, clinician training, and the Code of Medical Ethics-2002.
MOM.4.b	CORE*	Sections 3, 4, and 5.2 address all seven minimum prescription elements, capital letters, no error-prone abbreviations, and the process for holding non-compliant prescriptions. Physical-separation of requirements is presented as a bullet list for assessor legibility.
MOM.4.c	Commitment	Sections 3 and 5.3 address ascertainment of drug allergies and previous adverse drug reactions before prescribing. This is also the first stop-work trigger in Section 6.
MOM.4.d	Excellence	Section 5.4 addresses the clinician assistance mechanism — electronic or physical — for drug interactions, food-drug interactions, therapeutic duplication, and dose adjustments.
MOM.4.e	CORE	Sections 3 and 5.5 address medication reconciliation at all three mandatory transition points: admission, transfer between wards or departments, and discharge, with documentation and

		handover communication.
MOM.4.f	CORE*	Sections 3 and 5.6 address the verbal-order process: written guidance naming who, when, and how; read-back; countersignature within 24 hours; and a list of drugs orderable verbally. This is also a stop-work trigger in Section 6.
MOM.4.g	Achievement	Section 5.7 addresses the monthly prescription audit with a representative sample, covering all required scope parameters.
MOM.4.h	Achievement	Section 5.8 addresses corrective or preventive action taken where appropriate based on audit findings, with records kept.

13. Required Records/Evidence Checklist

Rational prescribing — MOM.4.a (Commitment*)

1. DTC-named rational-prescribing reference (Code of Medical Ethics-2002; applicable rational-prescribing guidelines).
2. Staff training records showing clinicians trained or sensitised on rational prescribing.
3. Prescription-audit records sampled against rational-prescribing guidance.

Minimum prescription requirements — MOM.4.b (CORE*)

1. Published minimum-prescription-requirement list showing all seven mandatory elements.
2. Sample prescriptions showing all elements present, written in capital letters, with no error-prone abbreviations.
3. Hold or reject records for prescriptions failing minimum requirements, except through the documented emergency path.

Drug-allergy check — MOM.4.c

1. Drug-allergy and previous-ADR ascertainment records before prescribing, including entries stating 'none known'.
2. Prominent allergy documentation in the medical record for outpatients and inpatients.

Prescribing assistance mechanism — MOM.4.d

1. Evidence of a working assistance mechanism — electronic or physical — for drug interactions, food-drug interactions, alcohol-drug interactions, therapeutic duplication, and dose adjustments.
2. Records of orders clarified by pharmacy before dispensing where the mechanism identified a concern.

Medication reconciliation — MOM.4.e (CORE)

1. Reconciled-medication-list records at admission (direct or post-emergency), transfer between wards or departments, and at discharge.
2. Discrepancy-resolution records between the reconciling clinician and the prescriber.
3. Handover communication records showing reconciliation status transmitted at the transition point.

Verbal orders — MOM.4.f (CORE*)

1. Written verbal-order guidance naming who can give a verbal order, when, and how it is authenticated.
2. Read-back records for each verbal order — drug, dose, route, frequency, patient — before administration.
3. Countersignature records by the ordering doctor within 24 hours of the verbal order.
4. Approved list of formulary drugs that can be ordered verbally.

Prescription audit — MOM.4.g

1. Monthly audit records with a representative sample, covering all required scope parameters.
2. DTC or Medication Safety Officer presentation record of audit findings.
3. Evidence the audit sample size was representative and the frequency was at least once a month.

CAPA from audit — MOM.4.h

1. CAPA records from audit findings, with owner and due date.
2. Closure-tracking records for each open action.
3. Decision records noting where CAPA was considered and judged not required, where that is the case.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.4.
2. National Medical Commission (adopting the Indian Medical Council). (2002). <https://www.nmc.org.in/wp-content/uploads/2017/10/Ethics-Regulations-2002.pdf> Code of Medical Ethics — Professional Conduct, Etiquette and Ethics Regulations.

3. Institute for Safe Medication Practices (ISMP) / ECRI. (n.d.).
<https://home.ecri.org/blogs/ismp-resources> List of Error-Prone Abbreviations (ISMP resources, now hosted by ECRI).

Disclaimer

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